Supplementary File 1(S1)

Logic Grid: **Pubmed: 153 : 29/06/2020**

HPV AND INDIGENOUS POPULATIONS

HPV	Indigenous Populations
HPV[Text Word] OR "papillomaviridae"[MeSH Terms] OR "human papillomavirus"[MeSH] OR "human papillomavirus"[Text Word] OR "human papillomavirus 16"[MeSH Terms] OR human papillomavirus 16 [Text Word] OR "human papillomavirus 18" [MeSH Terms] OR human papillomavirus 18 [Text Word] OR papillomavirus[Text Word]	("first nation"[Text Word]) OR "first nations"[Text Word]) OR "pacific islander"[Text Word]) OR "pacific islanders"[Text Word]) OR "torres strait islander"[Text Word]) OR "torres strait islanders"[Text Word]) OR aborigin*[Text Word]) OR africa*[Text Word]) OR alaska*[Text Word]) OR aleut*[Text Word]) OR amerind*[Text Word]) OR arctic[Text Word]) OR aymara[Text Word]) OR bushmen[Text Word]) OR chukchi[Text Word]) OR chukotka*[Text Word]) OR circumpolar[Text Word]) OR eskimo*[Text Word]) OR greenland*[Text Word]) OR hmong[Text Word]) OR indian*[Text Word]) OR indigen*[Text Word]) OR inuit*[Text Word]) OR inupiaq[Text Word]) OR inupiat[Text Word]) OR khanty[Text Word]) OR maori*[Text Word]) OR mapuche[Text Word]) OR metis[Text Word]) OR native*[Text Word]) OR navaho*[Text Word]) OR navajo*[Text Word]) OR nenets[Text Word]) OR quechua[Text Word]) OR saami[Text Word]) OR sami[Text Word]) OR samoan*[Text Word]) OR siberia*[Text Word]) OR skolt[Text Word]) OR tribal[Text Word]) OR tribe*[Text Word]) OR xingu*[Text Word]) OR yup'ik[Text Word]) OR yupik[Text Word]) OR zuni[Text Word]) OR "African continental ancestry group"[Mesh]) OR "African continental ancestry group"[Mesh]) OR "Asian continental ancestry group"[Mesh]) OR "Health Services, Indigenous"[Mesh]) OR "Oceanic ancestry group"[Mesh]) OR "arctic regions"[Mesh]) OR "ethnic groups"[mesh]

Logic Grid: **EMBASE: 271 : 29/06/2020**

HPV	Indigenous Populations
('human papillomavirus type 16':ti,ab,kw OR 'human papillomavirus type 18':ti,ab,kw OR 'wart virus':ti,ab,kw OR 'oral human papillomavirus infection':ti,ab,kw OR 'hpv':ti,ab,kw)	'indigenous population':ti,ab,kw OR 'indigenous people':ti,ab,kw OR 'Samoan (people)':ti,ab,kw OR 'First Nation':ti,ab,kw OR 'Pacific Islander':ti,ab,kw OR 'Torres Strait Islander':ti,ab,kw OR 'Black person':ti,ab,kw OR 'Alaska Native':ti,ab,kw OR 'Aleut (people)':ti,ab,kw OR 'Amerind people':ti,ab,kw OR 'Aymara (people)':ti,ab,kw OR 'Chukchi (people)':ti,ab,kw OR 'Chukotka':ti,ab,kw OR 'Eskimo-Aleut people':ti,kw,ab OR 'Greenland':ti,ab,kw OR 'Hmong (people)':ti,kw,ab OR 'Indian':ti,ab,kw OR 'indigenous people':ti,kw,ab OR 'Inuit':ti,kw,ab OR 'Inupiat (people)':ti,kw,ab OR 'Khanty (people)':ti,ab,kw OR 'Maori (people)':ti,kw,ab OR 'Mapuche (people)':ti,kw,ab OR 'indigenous people':ti,kw,ab OR 'Navajo (people)':ti,kw,ab OR 'Nenets (people)':ti,kw,ab OR 'Quechua (people)':ti,kw,ab OR 'Sami (people)':ti,kw,ab OR 'Yupik (people)':ti,ab,kw OR 'Zuni (people)':ti,ab,kw OR 'Asian continental ancestry group':ti,ab,kw OR 'Oceanic ancestry group':ti,ab,kw OR 'Arctic':ti,ab,kw OR 'ethnic group':ti,kw,ab

Logic Grid **SCOPUS: 764 : 29/06/2020**

HPV	Indigenous Populations
TITLE-ABS-KEY (HPV) OR TITLE-ABS-KEY (papillomaviridae) OR TITLE-ABS-KEY (human papillomavirus) OR TITLE-ABS-KEY (human papillomavirus type 16) OR TITLE-ABS-KEY (human papillomavirus type 18)	TITLE-ABS-KEY (indigenous AND population) OR TITLE-ABS-KEY (ethnic AND groups) OR TITLE-ABS-KEY (aborigin) OR TITLE-ABS-KEY (torres AND strait AND islander) OR TITLE-ABS-KEY (black AND person)

Web of Science: 189 :29/06/2020

HPV	Indigenous Populations
(HPV OR human papillomavirus OR human papillomavirus type 16 OR human papillomavirus type 18 OR Oral HPV OR wart virus)	**TOPIC:** ((first nation OR first nations OR pacific islander OR pacific islanders OR torres strait islander OR torres strait islanders OR aborigin* OR africa* OR alaska* OR aleut* OR amerind* OR arctic OR aymara OR bushmen OR chukchi OR chukotka* OR circumpolar OR eskimo* OR greenland* OR hmong OR indian* OR indigen* OR inuit* OR inupiaq OR inupiat OR khanty OR maori* OR mapuche OR metis OR native* OR navaho* OR navajo* OR nenets OR quechua OR saami OR sami OR samoan* OR siberia* OR skolt OR tribal OR tribe* OR xingu* OR yup'ik OR yupik OR zuni OR African continental ancestry group OR African continental ancestry group OR Asian continental ancestry group OR Health Services, Indigenous OR Oceanic ancestry group OR arctic regions OR ethnic groups))

S2: Supplementary File 2: JBI Summary appraisal tool questionnaire for qualitative systematic reviews

1. Is there congruity between the stated philosophical perspective and research methodology?
2. Is there congruity between the research methodology and the research question or objectives?
3. Is there congruity between the research methodology and the methods used to collect the data?
4. Is there congruity between the research methodology and the representation and analysis of data?
5. Is there congruity between the research methodology and the interpretation of results?
6. Is there a statement placing the researcher culturally or theoretically?
7. Is the influence of the research on the researcher and vice versa, addressed?
8. Are the participants and their voices adequately represented?
9. Is there research ethical according to current criteria or, for recent studies, and there evidence for ethical approval by an appropriate body?
10. Do the conclusions drawn in the research report flow from the analysis, or interpretation of the data?

S3: Supplementary file 3: Appraisal of included studies

Table: Qualitative Research

Citation	Q1	Q2	Q3	Q4	Q5	Q6	Q7	Q8	Q9	Q10
Adcock A. 2019.	Y	Y	Y	Y	Y	N	N	Y	Y	Y
Allen-Leigh B et al. 2017.	Y	Y	Y	Y	Y	N	U	Y	Y	Y
Cerigo H et al. 2012.	Y	Y	Y	Y	Y	N	N	Y	Y	Y
Cui Y et al. 2010.	Y	Y	Y	Y	Y	N	N	N	Y	Y
Henderson R et al. 2018.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
Maar N et al. 2016.	Y	Y	Y	Y	U	Y	U	Y	Y	Y
O'Brien B et al. 2009.	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
Tratt E et al. 2020.	Y	Y	Y	Y	Y	Y	U	Y	Y	Y
Wakewich P et al. 2015.	Y	Y	Y	Y	Y	Y	N	Y	Y	Y
Zehbe et al. 2017.	Y	Y	Y	Y	Y	Y	N	Y	Y	Y
%	100.0	100.0	100.0	100.0	90.9	63.63	18.18	90.9	100.0	100.0

S4: Supplementary file: List of study findings with illustrations

Study: Tratt E et al 2020	
Finding	Cultural Awareness (C)
Illustration	Cultural awareness of healthcare workers was a major facilitator, reflected in positive patient experiences and more use of adequate communication means. (Pg 3)
Finding	Previous Experiences (C)
Illustration	Previous positive experiences of health personnel in their interaction with Inuit was told to be a factor that highly contributes to increase cultural awareness, countering the negative effect of a healthcare system based on non-Inuit cultural values. (Pg 4)
Finding	Increased confidence in patients' capabilities (C)
Illustration	Thus, allowing a more active role of patient in decision-making. (Pg 4)
Finding	Access to medical information (C)
Illustration	The vast majority of participating women expressed the need to get screened after understanding medical facts, including but not limited to the absence of symptoms (Pg 4)
Finding	Personal Preference (U)
Illustration	"For me, I am used to getting my Pap, I'm fine with it. But for the younger girls, [or] the shy ones, [or] those who were sexually abused, they might prefer [the HPV self-sampling method]." (Pg 4)
Finding	Means of communication (C)
Illustration	Participants reported the major importance of using visual language, ranking higher than using Inuktitut, and they stressed the communication problems behind using French or medical jargon. (Pg 4)
Study: O'Brien B et al 2009	
Finding	Vivid Recollection of healthcare encounters (U)
Illustration	"My appointment was scheduled when the offices are closed so I knew he was going to be giving me some fairly bad news. The office was empty, and the lights were all turned down. It was a real comfortable surrounding. He gave me a lot of information, pamphlets, and people to phone for support." (Pg 85)
Finding	Not Knowing what was happening (inadequate explanations) (U)

Illustration	“I still don’t know, to this day, whether they found anything or what portion they removed, if they even removed anything. I don’t know! Nobody explained anything to me because, maybe because I didn’t ask. Maybe they thought that I understood everything that was going on, but I didn’t.” (Pg 86)
Finding	Not knowing what was happening (Confusion) (U)
Illustration	“When you’re in (western Canadian city A) and (western Canadian city B) sometimes you just get pushed along as if you’re just a file. “Here you go, this is what’s wrong, this is what we’re going to do, see you later.” I don’t understand where I go from there, how’s my health going to be affected by it now.” (Pg 86)
Finding	Encouragement and Support (Fear of process) (U)
Illustration	"My mom has gone through the process. She told me that she had cancer in her cervix and that she had to go for surgery. So it was a big fear for me because as soon as you hear cancer you think of the ultimate. There’s nothing more then; but for this person to die. "(Pg 86) "My dad had a really difficult time with it, because he was already in his early 60s at the time, and as soon as he heard cancer [he thought] I was going to die. He started cutting off the emotional ties with me then." (Pg 86) "People are scared. I’m thinking of my sister. She hasn’t gone back to get retested. She did have an abnormal result the first time. I think she’s afraid. Some people are afraid that if they do find cancer, it’s downhill from there." (Pg 86) "To find out that I have some kind of illness . . . like to know you’re sick you just want to block it out. . . . If I knew I had an illness . . . I would probably just shut down from everything and totally change, and not for the better either." (Pg 86)
Finding	Encouragement and Support (Motivation) (U)
Illustration	"My fear is of having [cervical cancer] . . . motivates me. I want to have good health. I don’t want that kind of a disease." (Pg 86)
Finding	Encouragement and Support (Support) (U)
Illustration	"I would try and keep encouraging her . . . to tell her, “If they find it early they can do something about it.” I try and set examples for my clients. I tell people, “Did you know this lady had this kind of cancer because she never got tested, and they could have done something about it right away and she would still be here.”" (Pg 86)
Finding	Encountering a male provider (Traditional Beliefs) (U)

Illustration	"I had it done by a male doctor and I was uncomfortable . . . because of the way I was raised. My [traditional] grandmother always told me that the only man that should know you like that is your husband." (Pg 87)
Finding	Encountering a male provider (Nervousness) (U)
Illustration	"My first Pap smear was done by a male and I was very nervous and I just didn't want it done. Maybe that's why I didn't have another one [Pap smear] . . . because of that [male gender]." (Pg 87)
Study: Maar N et al 2016	
Finding	(Explain the value of cervical screening) Importance & Benefits (U)
Illustration	"Explain [what the Pap test is] then they accept the value of early diagnosis: I find that once I explain to them the importance of early detection and [that] it can be treated, they're more agreeable to [a Pap test]." (Pg 4)
Finding	(Explain the value of cervical cancer screening) Awareness of screening services (U)
Illustration	"The biggest motivation is education, I think. You know, just teach them, let them know that the service is there, that they need to take it, it's important that they have it." (Pg 4)
Finding	(Explain the value of cervical cancer screening) Screening can prevent cancer (U)
Illustration	"We know cervical cancer is 100% preventable, and I know because I read stats and see statistics that Aboriginal women are the number one on the list for dying from this." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Build local HCP capacity (U)
Illustration	"You need to make sure that you train the trainers, like teach the CHRs [community health representatives], and the women in the community that are providing service." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Education by community cancer survivors (U)
Illustration	"I think if there was someone just sharing stories. If a woman was willing to share her story that she's had this and got screened early. Girls like to listen to things like that, and the women like to listen to things like that. I beat this and I did that. Something like that would motivate them." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Education by extended female relatives (U)

Illustration	"Well, my mother, you know, always went for her physicals; thus I learned that I was to go for my physicals... the families that do have it, you can see that progression of preventative health care." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Early education in schools (U)
Illustration	"Start early, in school already: we should get our younger girls out there, and have that part as [education], at [grade] 8, and I know they're doing great in the school, getting it out there, about STDs." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Regular workshops (U)
Illustration	"Workshops, information sessions: I have lunch and learns. That's a start. There's all different venues bringing that into the community." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Passive intervention for whole community (U)
Illustration	"You could even [do] something as simple as Facebook. Everybody's on Facebook." (Pg 5)
Finding	(Motivate through specific educational strategies and capacity building) Include humour in health education (U)
Illustration	"We didn't specifically talk about HPV, but we talked about STDs [during the education session]. Yeah, I think they got it because they still, when they see me on the streets here, we kind of giggle about it because we used bananas [for the sex education], you know, and they'll ask me: "Are you bringing bananas next week?" You know, so, it was a fun thing." (Pg 6)
Finding	(Motivate through specific educational strategies and capacity building) Use local language (U)
Illustration	"I have a sweet little old lady, she's probably close to 80; she won't speak it, she won't speak English, she understands you, she can speak English, but she would rather speak in her own language." (Pg 6)
Finding	(Embed cervical cancer screening into community events) Integrate screening into wellness events (U)
Illustration	"So if you're having something that they're able to come and access, [like a] sewing circle, or community kitchen, something that they're gonna get, then they'll come." (Pg 6)
Finding	(Embed cervical cancer screening into community events) Screening on days that honour women (U)

Illustration	"I would concentrate on a day that's important to the women, like Mother's Day. The mothers would all come and the grandmas would come and the aunts would come. I would have guest speakers come in as well and there would be a dinner, a luncheon, or a feast of some sort. I would bring somebody in who had a little bit of charisma like a Tai Chi instructor who cooks meals" (Pg 6)
Finding	(Embed cervical cancer screening into community events) Small incentives (U)
Illustration	"It's hard to get people coming unless we have food and incentives." (Pg 6)
Finding	(Health care providers need a trusting rapport with women) Friendly Relationship (U)
Illustration	"The women are, you know, fairly open once they're comfortable." (Pg 6)
Finding	(Health care providers need a trusting rapport with women) Trusting Relationship (U)
Illustration	"It's all about trust and relationships, like if you can't build a relationship you're not gonna get anywhere, you might not even see them again." (Pg 7)
Finding	Integrate cervical cancer screening recruitment into existing health care services (U)
Illustration	"Well, since you're here, you know, we haven't screened for this, and when was your last Pap and what about colon cancer screening?" (Pg 7)
Finding	(Create a dialogue on cervical cancer screening in day-to-day activities) Sustain screening culture with HCP (U)
Illustration	"For our little group of work girls, [the] service team, we always tend to remind each other as well, like, "Hey, is it time for your check-up yet, like?"" (Pg 7)
Finding	(Create a dialogue on cervical cancer screening in day-to-day activities) Word of mouth reminders (U)
Illustration	"I think it's just sort of word of mouth, spreading things around, and just kind of bringing it up, you know, all the time." (p. 7) "I call four people and then each one of the people I call, they call four people and it's something that we're trying to work on." (Pg 7)
Finding	(Incorporate First Nations gender perspective on body and preferred screening techniques to enhance privacy and comfort) Screening as a way for women to re-gain teaching roles (C)

Illustration	"The regular program for girls meets every second Monday, and it's called the Moon Time Girls." (Pg 7)
Finding	(Incorporate First Nations gender perspective on body and preferred screening techniques to enhance privacy and comfort) Holistic approach for cultural integrity (U)
Illustration	"Well, word it a way to have to do with childbearing, [which] is a sacred gift that's only given to women, let's keep our bodies healthy by getting annual check-ups, and you use some culturally appropriate like pictures, or something." (Pg 8)
Finding	(Incorporate First Nations gender perspective on body and preferred screening techniques to enhance privacy and comfort) Using non-invasive screening (U)
Illustration	"Informant #14: I think that's the best idea that has come out of anything, like it's not invasive, you do it in your own home, at your own time... I.Z.: Whenever you, you know, are ready for it and... Informant #14: I think that with education along with it that this is what it is, I think that the success rate will be phenomenal with it because it's in their own home." (Pg 8)
Finding	(Incorporate First Nations gender perspective on body and preferred screening techniques to enhance privacy and comfort) Multi-purpose screening (U)
Illustration	"If it [other STI testing] can be done on the same swab [as self-sampling], I would say, the value of that would be huge." (Pg 8)
Finding	(Incorporate First Nations gender perspective on body and preferred screening techniques to enhance privacy and comfort) Preference of female HCP (U)
Illustration	"The girls are always looking for female doctors" (Pg 8)
Study: Allen-Leigh B et al 2017	
Finding	Beliefs and knowledge about cervical cancer (Correct knowledge) (U)
Illustration	"If you detect it in time, you can take that sickness out so it won't progress any more and with a treatment you are fine." (Pg 4)
Finding	Beliefs and knowledge about cervical cancer (Incorrect knowledge) (U)
Illustration	"Between these two sicknesses [HPV and cervical cancer] we're in danger, we should go to the clinic or a doctor. If we feel pain in the womb, go to a doctor." (Pg 4)
Finding	Perceptions of and knowledge about HPV (Correct knowledge) (U)

Illustration	[Cervical cancer is caused] "by infections, because later they become like a tumor in the womb." (Pg 4) "There is a vaccine for human papillomavirus. I heard it on the radio, that there are vaccines." (Pg 4)
Finding	Perceptions of and knowledge about HPV (Combined correct and incorrect knowledge) (U)
Illustration	"Well, as far as I know, the virus is transmitted through sexual contact. Then this human papillomavirus, it begins without any warning. Then later it progresses, then the discomfort in our parts [genitals] begins and that's when the discharge starts and it progresses to the cervix and when it gets to the cervix it goes into the uterus and that is when the doctor sends us for an operation." (Pg 4)
Finding	Perceptions, knowledge and experiences related to cytology (Combined correct and incorrect knowledge) (U)
Illustration	"Moderator: What is cervical cancer, what do you know about cervical cancer? Huichol woman 1: They say you can die of cancer, if you don't detect it early. Moderator: And how do you detect it? Huichol woman 1: With the Papanicolaou, doing it periodically. Moderator: And what's periodically? Huichol woman 1: Every three months. Moderator: Everyone, how often do you think you need to get a Papanicolaou? Huichol woman 2: Once a year. Huichol woman 3: Depends on how you feel, once a year or every two years, I get it every two years. Moderator: What do you mean, how you feel? Huichol woman 3: If you feel burning." (Pg 4)
Finding	Barriers to cervical cancer screening using cytology or the HPV self-sampled test (Pain and fear of unsterile equipment as a barrier) (U)
Illustration	"[When they did the Papanicolaou test], maybe they did it wrong. I don't know, but I felt something like a scrape. Then I thought maybe the equipment wasn't disinfected. ... I thought about it a lot before doing it again, because I was afraid." (Pg 4)
Finding	Barriers to cervical cancer screening using cytology or the HPV self-sampled test (Not receiving test results as a barrier) (U)
Illustration	"A lot of us say, and we've talked about this before, when we get a Papanicolaou, the results don't arrive, and we don't know what it is that's going on. There we are with the doctor, asking why the results don't arrive. ... Whatever it [the result] is, they should give it to me." (Pg 4)
Finding	Barriers to cervical cancer screening using cytology or the HPV self-sampled test (Unequal gender relations as a barrier) (U)

Illustration	"The first time I went to check myself, with the Papanicolaou tests, I had problems. I got beat-up. My husband hit me because he said I had gone to do things with the [male] doctor. When it wasn't even a doctor who examined me, the [female] nurse examined me! She took the sample, but at home my husband didn't believe that." (Pg 4)
Finding	Barriers to cervical cancer screening using cytology or the HPV self-sampled test (Lack of gender-related barriers) (U)
Illustration	"Because with all these [health education] talks they give us, women are more secure in themselves. So, we don't really ask men for permission anymore, because it's something that's good for us." (Pg 4) "I decided it [to perform the HPV test] myself, alone. I don't ask anyone's permission. ... How am I going to ask him if he [her husband] wants it or not? It's not for him, it's for me." (Pg 4)
Finding	Perceived advantages of the self-sampled HPV test (Less embarrassment) (U)
Illustration	"This one [the HPV test] is good because it [cytology or the Papanicolaou test] really does embarrass you, not because your husband doesn't want it or doesn't let us, but because it's embarrassing and because of embarrassment we don't do it, and so this [self-sampled HPV test] is good for us." (Pg 5) "Because you do it yourself, since always, even if there is trust, you feel a little embarrassed to undress in front of someone else" (Pg 5)
Finding	Perceived advantages of the self-sampled HPV test (More comfortable) (U)
Illustration	"Well, this one [the self-sampled HPV test] is better, because it is more comfortable to do it." (Pg 5)
Study: Cerigo H et al 2012	
Finding	Experiences, Attitudes and beliefs about cervical cancer (C)
Illustration	A sizable proportion of the women were unable to identify a cervical cancer risk factor and were unsure if detecting cervical cancer early would affect the chance for a cure. (Pg 5)
Finding	Pap smear history, understanding and experiences (female preference) (U)
Illustration	""Some people don't go [to get Pap smears], maybe they're shy … all of my friends don't go probably because they are shy or sometimes they don't want to [be] checked by a man, if it is a man that is a nurse." (Pg 3)

Finding	Pap smear history, understanding and experiences (embarrassment) (C)
Illustration	Feelings of embarrassment and pain during Pap smears were common among our study population and others. Among our population, older women reported more feelings of embarrassment than the young women. (Pg 5)
Finding	Pap smear history, understanding and experiences (lack of knowledge) (C)
Illustration	Further, we found that some women did not fully understand the purpose of the Pap smear as a method of cervical cancer screening, as 30% reported that the purpose of the Pap smear test is to screen for STIs. (Pg 5)
Study: Zehbe et al 2017	
Finding	Educational implications (contribution to stereotype) (U)
Illustration	Focus group participants were concerned that any publication of information about HPV rates on reserve could have negative effects, contributing to the stereotype that 'all native people have HPV' (FG6). As one HCP summarised: 'A lot more education has to be done about HPV and cervical cancer together' to alleviate these concerns (Pg 7)
Finding	Educational implications (women blamed for STI's) (U)
Illustration	"'HPV, like, whoa, I don't have that, like, I don't even want to know if I have that" (Pg 7) 'because it's a relationship thing' that implied that someone had 'cheated' (Pg 7) '…. do they [the men] not need to know some of this stuff too?' (Pg 7) '… I know only women get cervical cancer but … they've helping it along in the same sense where they've passing it back and forth' (Pg 7)
Finding	Educational implications (need to explain self-sampling) (U)
Illustration	HCPs felt that education around self-testing might better be done in the context of a well woman healthcare visit, as 'part of a physical, that might get best [results]' (Pg 7) Women would not necessarily understand that they would still need to go to their HCP for other reproductive healthcare: 'They would, they'd probably think, well, I've had that done, so I don't need this done'" (Pg 7) "If you hand me a kit, I won't touch it. Because I wouldn't know what to do, what if I did it wrong, or whatever, right, so? And I think a lot of

	people would be that way" (pg 7) 'you just give a test kit to them at home; the majority of them are just going to throw them out.'(Pg 7)
Finding	Convenience factor self-sampling (no appointment needed) (U)
Illustration	'It is more comfortable to do it at home … it's simple'. (Pg 6) I think the self-test is beneficial for them all because sometimes people don't have time for appointments to take off work, it [the Pap] is kind of an inconvenience' (Pg 6) 'would go for it' (Pg 6) older woman who had fewer constraints 'were used to getting Paps' (Pg 6) "Even hearing [about self-testing], people are just, What? Oh, I'd do that, for sure, instead of me going to the doctor' " (Pg 6) "Doing a self-test doesn't take very long … it's something that can be dealt with, done, gone' (Pg 6) " I really strongly believe that … because I was the one that was doing it [self-sampling], I was the one that was in control … and this way it gave me the ability to do it myself and I got all the results, they were fine; … it was also self-empowering, great, I like that." (Pg 6)
Finding	Convenience factor self-sampling (more privacy) (U)
Illustration	"I think more people would monitor it that way and test it themselves, like 'well maybe not today, but … eventually I'm going to try it' (Pg 6) option of taking it home might make women 'feel more empowered' and contribute to a better relationship with HCPs. (Pg 6) An option, they can either do it themselves, or you can offer to do it for them' (Pg 6)
Finding	Psychological comfort (high comfort with self-sampling) (U)
Illustration	"doing it yourself would be better" "would feel more comfortable with that [self-test] than a male doctor" (Pg 6) 'shy' of doctors or nurses who visited the communities. (Pg 6) Self-testing would also address the 'trust issues' that discouraged women from seeking care from non-indigenous HCPs. (Pg 6)
Finding	Psychological comfort (male providers) (U)
Illustration	"feel more comfortable [with a female physician or nurse practitioner] because maybe she's going through the same thing that you're going through." (Pg 6)

	"embarrassed anyway, no matter who did it (Pg 6) "I only got a Pap test once and I got it from a woman … and she was trying to make me feel comfortable, but it's still really awkward" (Pg 6)
Finding	Physical comfort level (self-sampling is less painful) (U)
Illustration	"Women wouldn't be so agitated and nervous about having the [self-] test'"(Pg 5) "It's a lot less clinical … stripping down and allowing someone else to do the scraping of the cervix, the whole uncomfortable procedure of going through a Pap opposed to doing it privately in the bathroom on your own is a huge difference.' '…I think the prevalence [participation] rate would go up' (Pg 5) "It's a lot less clinical … stripping down and allowing someone else to do the scraping of the cervix, the whole uncomfortable procedure of going through a Pap opposed to doing it privately in the bathroom on your own is a huge difference.' '…I think the prevalence [participation] rate would go up'" (Pg 5)
Finding	Physical comfort level (paps hurt) (U)
Illustration	"it's kind of a routine after a while … they open you up and they swab and then you're done" (Pg 5) "the Pap is uncomfortable but it has to be done" (Pg 5) "it doesn't get any easier, like the first time and then the next year, it didn't get any easier for me" (Pg 5) "I didn't like the way it felt, that's why I didn't want to go back there' They don't want their Paps [be]cause it hurt." (Pg 5) The test also could be exceptionally painful if providers were 'in a rush' or 'rough' and did the procedure 'real quick' (Pg 5) "Some women, when they come to the appointment, they decide they don't want to get a Pap, because it's uncomfortable, they're just afraid" (Pg 5)
Finding	Cervical Screening practices in partner communities (privacy concerns) (U)
Illustration	"health centres where staff 'phoning people, telling them, 'we've got your results … you need to make another appointment' (FG2) was upsetting." (Pg 5) "I don't want nobody else to know' 'a lot more private, at home, if you do it by yourself"(Pg 5)

	"Women would prefer to be discreet, do it themselves and get their own results and not have their results shared with others" (Pg 5)
Finding	Cervical Screening practices in partner communities (no efficient follow up) (U)
Illustration	"It's crazy here, how long you have to wait for anything and half the time, you don't even get a call … you go to your next doctor's appointment for something else and [have to ask] … by the way, how about my Pap two months ago?" (Pg 5)
Finding	Cervical Screening practices in partner communities (transportation and child care issues) (U)
Illustration	"If the van was full, [you would] have to get a ride … some of us don't have cars, you know" (Pg 5) Women with children and work commitments also had difficulty arranging childcare and time off work to keep their appointments. As a HCP commented: 'It certainly does pose challenges with regards to babysitting care and the mother being away for an entire day' (Pg 5)
Finding	Cervical Screening practices in partner communities (lack of provider flexibility) (U)
Illustration	"I don't know how many times they've cancelled Pap tests and [when I asked] "okay well can I get it on Friday? …. [heard back] "No, we don't do them on Friday"' (Pg 5)
Finding	Cervical Screening practices in partner communities (no regular screening service) (U)
Illustration	"'Most of us do not have doctors here … you have to drive up to [nearby town]'" (Pg 4)
Study: Adcock A 2019	
Finding	Engagement with health services (C)
Illustration	One in seven (14.36%) had not screened in ten years or more (including those never screened), and one in six (16.37%) were unsure how long it had been since their last cervical screen. (Pg 2)
Finding	Barriers to screening (shyness, embarrassment) (C)
Illustration	Desire for bodily autonomy (retaining privacy, control over one's body) as a reason for not attending regular cervical screening. (Pg 3) encompasses concepts of whakamā (embarrassment/shyness/reticence), (Pg 3) tapu (sacred/taboo/forbidden) (Pg 3)

	desire for bodily autonomy was often related to negative health experiences (eg, painful pelvic examinations, inappropriate actions/comments by HCPs) (Pg 3)
Finding	Barriers to screening (cost) (C)
Illustration	Older women were more likely to mention a previous bad experience and were less likely than younger women to mention cost or other financial barriers (Pg 3) Opinions varied about cost being a barrier (as many clinics offer free cervical screening). (Pg 3) Some highlighted hidden costs (transport, parking, childcare) (Pg 3)
Finding	Barriers to screening (education) (C)
Illustration	A lack of health literacy about HPV and cervical cancer and a lack of appropriate/empathetic services were also raised as barriers. (Pg 3) relationship-building between communities and health promoters; including whānau/family in HPV education; and ensuring clear information about HPV vaccination (Pg 3) HCPs agreed that with appropriate support and education, HPV self-testing will benefit never/under-screened Māori. (Pg 4)
Finding	Acceptability (U)
Illustration	positive, with participants using terms such as ‘easier’, ‘more comfortable’, ‘less intrusive’ and ‘brilliant’. (Pg 3) Nearly two-thirds (61.21%) said they would prefer an HPV self-test to a clinician-collected vaginal swab or a cervical sample collected with a speculum, or to other options. (Pg 4)
Finding	Implementation (C)
Illustration	picking up the HPV self-test from a clinic, a pharmacy or other community venue, or receiving it by post/mail (Pg 4) concerns were raised about the reliability of post. Participants discussed the value of providing multiple options (Pg 4) Flexibility to cater to diverse populations, such as through community outreach services, was suggested for optimum engagement. (Pg 5)
Finding	Doing the self-test? (U)
Illustration	most frequently said they would be happy to do the HPV self-test in a clinic or their own home, and emphasised having good support and education to increase their confidence about properly doing it. (Pg 5) need for a flexible program, with different options. (Pg 5)

	Urban women were more likely (than rural) to say they would self-test at a community centre. (Pg 5) One in five said they would like whānau/family or friends to be around in case they needed help, while some (<15%) said they would like either a community health worker or health practitioner to be nearby, available to help. (Pg 5) importance of delivering empathetic cervical screening services, ie 'the process' – taking the time to put mind and body at ease. They emphasised that this process will be important for any new HPV-based cervical screening program. (Pg 5)
Finding	HPV self-test results (C)
Illustration	if negative a text is fine, but if positive it is better to find out in person or by phone. (Pg 5) One women emphasised the importance of results being delivered in an empathetic way.(Pg 5) Older women were less likely than younger women to want to receive a text asking them to call their health practitioner.(Pg 5) Non-PHO-enrolled/unsure women were less likely to choose to receive results by email and more likely to choose a visit with a health practitioner need for clear communication (Pg 5) appropriate/empathetic support for women. (Pg 5) Concerns were raised about who would be responsible for HPV self-test results and making sure that women are added to the screening register when doing a self-test.(Pg 5)
Finding	Positive test results (C)
Illustration	mostly positive response to the idea of seeking further screening (cytology) or diagnosis (colposcopy) if a HPV test was positive – because knowing that something needs follow up would be a good motivator.(Pg 5) HCPs wanted assurance that women would be supported appropriately and empathetically to have cytology or colposcopy if they had a positive HPV self-test result, and many supported going straight to colposcopy.(Pg 5) Eliminating multiple clinic visits was stressed. (g 5)
Study: Henderson R et al 2018	
Finding	(Cancer & HPV Experiences) Disease Progression (U)
Illustration	"Once they corrected the cervical cancer you would end up with cancer somewhere else" (Pg 96)

Finding	(Cancer & HPV Experiences) Isolating Effects (U)
Illustration	"It took time for one family to realize that it was 'not just happening to them, it's the whole community'" (Pg 96)
Finding	(Cancer & HPV Experiences) Mistrust of external influences (U)
Illustration	"It must be the pesticides that are being sprayed on our cultivated land...You can see our area and there is a layer of dust in spring and harvest" (Pg 96)
Finding	(Cancer & HPV Experiences) Residential school disruption of community relationships (U)
Illustration	"Opened a whole can of worms; when all the illness and substance abuse went on a skyrocket" (Pg 96) "Speaking of a sister in the end-stage of cervical cancer, one middle-aged participant attributed the illness to chronic sexual abuse: "There were a couple times she had STIs [sexually transmitted infections] and didn't really know, may have been HPV or may have been another" (Pg 96) "In both sharing circles, participants argued that residential school litigation broke students' capacity for intimacy throughout their lives, playing out in self-destructive coping strategies (e.g., substance dependence, early sexualisation) and increased risk of victimization)" (Pg 96)
Finding	(Trauma-informed lens) Contributions of higher Indigenous HPV burden (U)
Illustration	"Our people are starving for affection, support, respect and love." (Pg 96)
Finding	(Trauma-informed lens) Residential schools (U)
Illustration	"The nuns make sure to remind you no matter what you do, you're going to hell" (Pg 96) "We were taught to be quiet about our private parts...a lot of sexual abuse went on, and spiritual, physical, you name it" (Pg 96) "One speaker brought these lingering effects of residential schools together by describing a shift in her society from monogamous relation- ships to relaxed sexual boundaries, noting that with her own daughters she "didn't lay down the law like our parents did" (Pg 96)
Finding	(Trauma-informed lens) Normalization of substance abuse among youth (U)
Illustration	"She got an STI because she was taken advantage of" (SC2) by someone other than her partner. The mother connected her daughter's

	exposure to loss of identity and heavy drinking at parties in her community. Lowered inhibitions brought on by alcohol, and illicit drug use were seen by several other speakers to have become normalized among younger generations, who "see the suffering of their parents and grandparents, and they are running to the substance abuse" (Pg 96) "In both circles, speakers expressed compassion for younger generations today: "It was a lot easier in my time because we would go to ceremonies...but today, our grandchildren are exposed to drugs, alcohol, everything" (Pg 96)
Finding	(Trauma-informed lens) Isolation (U)
Illustration	He observed that such statistics reflect disrupted connections between partners, between Elders and youth, as well as with nature and spirituality: "We are not islands, we need to be connected to people and that is what is missing" (Pg 96) "Therefore, the burden of HPV in FN people is rooted, at least in part, in efforts by people across the lifespan to cope with the violent disruption of family and community connectedness, as well as connection to land and spirituality. This burden is manifested not only in risk-taking behaviour, but also in avoidance of wider health systems: "I think First Nations don't get checked when they are supposed to, to be honest. They just wait until it is too late to help them" (Pg 97) Coming from a communal society, another participant observed that today many struggle "to fit into an individualistic society, and we don't fit" (Pg 98)
Finding	(Trauma-Informed lens) Child welfare (U)
Illustration	"A lot of those kids when they come back to the reserve they are very angry, there are a lot of things that happened [to them] that we don't know about" (Pg 97) "It's sad to say that lots of babies have been taken because their mothers were on opium, and the mothers are in treatment and are so sorry" (Pg 97)
Finding	(Family & Community) Communication (U)
Illustration	"I am learning a lot in these workshops. My mother died of stomach cancer, my sister of stomach cancer. I had 5 girls, and 4 of them went through breast cancer. My oldest daughter, her cancer spread. When they were younger, I made sure they all got their needles. But, you know, I have never had a workshop like this. If I get a cold, I can fight it off. When I got those needles, I was told I was able to fight the sicknesses; it won't kill you—that is what I was told. This is really good for my grandchildren; I will take this message home to my

	family. I have two nurses in my family, they probably know about it, but this is a really good thing I am still learning...[*speaking in Cree] *I was worried the white people would not take care of us, but they have so far [group laughs]. We need to talk to young ladies about how to take care of themselves." (Pg 97)
Finding	(Family & Community) Family (U)
Illustration	Many of the Elders emphasized that health education within Indigenous contexts is anchored in the love and care for children. For one speaker, this affection involves "hugging your children and telling them 'I love you', you don't say goodbye to anyone" (Pg 98) "If I'm drinking and drugging, then I'm going to feel guilty about taking my children to the clinic to help them live a healthier life" (Pg 98)
Finding	(Family & Community) Role of elders (U)
Illustration	"[My husband] goes and sits with the men and teaches them; for me, I can go out and explain things to the mothers and the children, and out of respect...he's getting that message across [in his sweat lodge ceremonies with men]" (Pg 98)
Finding	(Family & Community) Mistrust in HCP (U)
Illustration	"We need to talk to our people in a way they understand, no disrespect to any organization, but people don't look at that material [brochures], it's obvious that the way we deliver that message has to be different" (Pg 98) "The health centre sent out a notification and a consent form, and they listed the benefits and risks...and I paid more attention to the risks, and I decided not to allow her to be vaccinated, because as a parent I needed to do what was best for my children" (Pg 98)
Finding	(Family & Community) Youth (U)
Illustration	"There are anti-bullying programs...and there are cultural programs; some kids are brave and some are afraid of getting immunized, but all the children support each other" (Pg 98)
Finding	(Family & community) Parents & grandparents (U)
Illustration	"The best teachers are your parents" (Pg 98)
Finding	(Changing information landscapes) Lack of knowledge (U)
Illustration	"We never see a doctor. The health department has to get a hold of us, the ones that are never home. There are a lot of us in the community... We hear suddenly of these workshops and information centres in the community. We wanted to go but we had other commitments so

	needed others to go. My niece, she died about 3 years ago because she had cancer. They brought her to Edmonton, they started her on chemo...left her kids at home." (Pg 97) “One of the girls I used to visit got cervical cancer. She is very angry; she will not talk to me even though I give her support. She was told 3 years ago that she should get the tests and she didn't because she had problems for a long time.” (Pg 97) One health director who was herself FN had not approved the HPV vaccine for her own daughter, believing at the time that vaccines are perhaps “not natural, that they are more chemicals given by the government to hurt us” (Pg 98)
Finding	(Changing information landscapes) new knowledge (U)
Illustration	“These young people are lucky to get these different resources...its scary when you think about it, you didn't think about [the health risk] before, because you didn't know anything about it” (Pg 98)
Study: Wakewich P et al 2015	
Finding	Negative body perceptions (stigma of sexually transmitted infections) (U)
Illustration	"Definitely there’s always going to be a stigma about any kind of thing that’s an STD [STI], because people think it’s dirty or whatever. Like I’ve had people come in, like, even the girl I just saw with genital warts and she just couldn’t believe it, and couldn’t like fathom who she would have got it from because everyone she’s been with has only been with her. Right? " (Pg 373) "Actually in the last 6 months that I’ve been there, I’ve had uh, a minimum of 5 people who have come to me who um, have been concerned about contracting STIs and did not want me writing it into their chart for fear that it would leak into the community … they’re afraid of the stigma surrounding that so and a lot of times too I wasn’t able to do any [cervical] screening." (Pg 373) "Health centres on reserves, don’t want to spread it out too much how many people have HPV. I know how the stereotype works if we pass on the information in non-native communities they will say all native people have HPV." (Pg 373)
Finding	Negative body perceptions (body shyness) (U)
Illustration	"I remember feeling so vulnerable, just so extremely vulnerable and, and I remember at one point he [the male physician] was talking on his phone while I was up in the stirrups and I thought uh, you know, I wonder how many other patients feel like this …. There’s got to be a better, kinder, gentler, more humane way to do it." (Pg 373)

Finding	Negative body perceptions (sexual abuse) (U)
Illustration	"They have been sexually abused, too, and I know like, in the past residential schools, that kind of thing, those people are just not comfortable because of their experiences in the past …. I will be here for a long time and whenever you need to see me, to come see me, so that even just that little thing and then when they do come I do see them, hopefully that trust builds up and I think that's a big piece with the First Nations." (Pg 374) "I think one of the biggest issues um, [short pause] the barriers that prevents people from going to and maybe it's cause it's taboo is because they've been sexually abused." (Pg 374)
Finding	Negative experience with governmental health service (distrust of health authorities) (U)
Illustration	"She [HCP] thinks it's her [that they don't like], 'like, why is no one coming [for screening]'… 'it's not you, it took me 20 years to get to where I'm at now, and I still get people that don't trust me.'" (Pg 374) "I guess they were told about some kind of vaccine or something, like years ago, and it was something just, to try to get rid of the Native people." (Pg 374) "My doctor who I had since my oldest was born, she just up and quit, so when my next appointment, I had this guy looking at me, it's like [pause] right, so it's not like they stick around here." (Pg 375)
Finding	Negative experience with government health service (lack of confidentiality and privacy) (U)
Illustration	"You know what, just sitting in the waiting room, the receptionists are phoning people telling them, we've got your results, you need an, you know, make another appointment, they say the name right out loud, they say what the test is." (Pg 375) "Privacy, a small town, I mean, you can hear through the walls, you know, walls talk because there's, it's all in one building and coming in here and everybody sees the first person coming in here, they come to see me, they come to see welfare, they come to see housing, they're going and everybody knows everything."(Pg 375) "Being in reserves, a lot of people know people's business and a lot of people get worried about that when you're trying to keep something personal. I mean the teddy bears talk, the leaves talk, the hydro lines talk." (Pg 375)
Finding	Role of family (mother - daughter relations) (U)
Illustration	I just taught them that sex isn't, it's not what you call dirty, you know, but there's a certain way you got to go about having, like for sex with

	a partner, you know, you got to explain that part to them, but to talk openly about sex, it's not what you call a dirty subject, yeah. If you want to know something just feel free to ask." (Pg 376) "Like my mother, you know, was the kind of woman that … when I first got my period, I didn't know what it was because that was something you don't talk about, your body like that. It's you know that's your own personal, private, so when it happened, I had absolutely no knowledge what was going on in my body … And with my daughter, I didn't want her to have that feeling, so I mean, of course I changed and I explained everything but, and then that's the way things are now, women are more informed than they have been in the past." (Pg 376)
Finding	Self-collected sampling as a way of increasing women's control of cervical screening (U)
Illustration	"think it [self-sampling] would be, I think it would be great, because it provides them with some autonomy and allows them to take control of the situation. That would be really great actually …. I think it would increase [education opportunities] actually, yup, because it's a lot less clinical right, cause like you say, it's a lot less you know, stripping down and you know, allowing someone else to do the scraping of the cervix, you know, the whole uncomfortable procedure of going through a Pap opposed to doing it privately in the bathroom on your own is a huge difference." (Pg 376) "I think it would definitely be more private … They wouldn't be so embarrassed … all kinds of people are easily embarrassed." (Pg 376) "It's simple, it's not like you are having forceps in you." (Pg 376)